PEARSON EDEXCEL GCSE (9–1) HISTORY • PAPER 1 OPTION 11

MEDIEVAL MEDICINE IN BRITAIN (C1250–C1500)

Supernatural Orthodoxy, The Galenic Monopoly & The Black Death



HISTORICAL OVERVIEW • C1250–C1500

In medieval Britain, illness was understood through Catholic theological dogma and ancient classical authority. The Church held an institutional monopoly over learning, scriptoria, and universities, declaring Claudius Galen infallible. Disease was interpreted as divine retribution for sin, celestial alignment, or humoural imbalance, while human dissection remained strictly prohibited and questioning doctrine was condemned as heresy.

When the catastrophic Black Death struck in 1348, killing over a third of the population, a society bound to the Four Humours and miasma theory stood powerless. From flagellants in city streets to herbal remedies, prayer, and bloodletting, this volume charts an age where religious devotion and Galenic tradition defined the human struggle against disease.

SPECIFICATION & ENQUIRY DIRECTORY

Pearson Edexcel GCSE (9–1) History

ENQUIRY 01 pp. 3–4

Supernatural & Religious Explanations of Disease

KEY QUESTION: *"Why did the Catholic Church hold an unshakeable monopoly over beliefs?"*

- **Ecclesiastical Monopoly:** Catholic Church controlled education, universities, and medical licensing, enforcing doctrine that illness was divine retribution for sin.
- **Astrological Alignment:** 1345 planetary conjunction of Saturn, Jupiter, and Mars in Aquarius blamed for corrupting air and bodily humours.
- **Galenic Teleology:** Galen's belief in a single divine Creator adopted as Church orthodoxy; questioning classical doctrine condemned as heresy.

ENQUIRY 02 pp. 5–6

Rational Explanations: Galen & The Four Humours

KEY QUESTION: *"How did the Four Humours provide a complete explanation for illness?"*

- **Humoural Equilibrium:** Health required balancing Blood, Phlegm, Yellow Bile, and Black Bile, linked to seasons and elemental qualities.
- **Theory of Opposites:** Galen extended Hippocrates by treating humoural excess with opposing qualities (e.g. cold phlegm treated with hot peppers).
- **Miasma Theory:** Rotting organic matter and swamp vapors believed to produce foul airborne miasma that disrupted internal humoural balance.

ENQUIRY 03 pp. 7–8

Approaches to Treatment & Prevention: Bleeding & Purging

KEY QUESTION: *"Why did standard treatments frequently endanger the lives of patients?"*

- **Phlebotomy & Purging:** Bloodletting via vein incision, cupping, or leeches guided by the Zodiac Man (Homo Signorum); violent emetics and laxatives.
- **Religious Penitence:** Confession, pilgrimages to holy shrines, prayer to patron saints, and wearing religious amulets to appease God.
- **Miasma Countermeasures:** Carrying pomanders stuffed with sweet herbs, burning aromatic incense, and ringing church bells to clear foul air.

ENQUIRY 04 pp. 9–10

Medical Care Providers & Monastic Hospitals: Care Not Cure

KEY QUESTION: *"How did social hierarchy and the Church shape medical care providers?"*

- **Physicians & Barber-Surgeons:** University-trained physicians examined urine charts for the rich; barber-surgeons performed bloodletting and minor surgery.
- **Monastic Hospitals:** Run by monks and nuns offering warmth, shelter, and prayer ("care not cure"); infectious and terminal patients strictly excluded.
- **Wise Women & Apothecaries:** Local herbalists provided traditional folk remedies in villages; apothecaries mixed herbal recipes and charms.

ENQUIRY 05 pp. 11–12

Case Study: Dealing with the Black Death (1348–1349)

KEY QUESTION: *"Why were medieval communities and authorities powerless to stop plague?"*

- **Arrival & Catastrophic Toll:** Plague landed at Melcombe Regis in 1348, wiping out 30% to 45% of England's total population within eighteen months.
- **Desperate Treatments:** Strapping live chickens to buboes, lancing swellings with hot irons, and public self-flagellation in city streets.
- **Civic Failure:** Quarantine attempts failed; authorities dug mass plague pits, shut city gates, but lacked understanding of flea or rat vectors.

THEMATIC MATRIX pp. 2–11

Thematic Matrix: Longitudinal Factors & Synthesis

KEY QUESTION: *"Which historical mechanisms dominated medicine across the Middle Ages?"*

- **The Catholic Church:** Maintained an absolute institutional monopoly on knowledge; preserved Galen's texts while strictly forbidding human dissection.
- **Continuity & Orthodoxy:** Humoural balancing and miasma theory created complete intellectual continuity, preventing any change in clinical therapy.
- **Government & War:** Weak royal government played minimal public health role beyond emergency Black Death quarantines and minor town ordinances.

SPECIFICATION PAPER	SECTION B QUESTION ARCHITECTURE	ASSESSMENT WEIGHTING
Paper 1 Option 11 • Thematic Study	Q3: Similarity/Diff [4m] • Q4: Causation [12m] • Q5/Q6: Essay [	36 Raw Marks + 4 SPaG • 70% of Paper 1 T

ENQUIRY 01 • PART I

Supernatural & Religious Explanations of Disease (c1250–c1500)

KEY ENQUIRY QUESTION *Why did the medieval Catholic Church hold an unshakeable monopoly over beliefs about the causes of disease?*

[1.1] Divine Retribution and Original Sin

"How did church scriptoria prevent new medical ideas from spreading?"

Between 1250 and 1500, the Roman Catholic Church was the supreme authority governing medieval thought and daily life. Ordinary peasants and learned scholars believed that disease was not merely a physical malfunction, but a direct manifestation of God's will. Deeply rooted in **original sin & human fallen state**, sickness was accepted as **divine punishment for sin** sent to test faith or purge wickedness.

Because suffering was viewed as spiritual chastisement, medieval society turned to religious intervention rather than physical science. The Church taught that enduring illness patiently on earth could shorten a soul's time in Purgatory. Consequently, whenever epidemics struck, communities relied on **prayer & repentance as primary cure**, organizing penitential street processions, strict fasting, and solemn confessions before seeking earthly herbal remedies.

[1.2] Moral Taint: Leprosy and Social Exclusion

"Why was leprosy viewed as visible proof of divine judgment?"

Few diseases illustrated the fusion of physical illness and spiritual judgment as vividly as leprosy. Lepers were believed to bear an outward physical sign of internal corruption and sinful lust, suffering intense **Biblical leprosy moral stigma**. Once diagnosed by parish priests or bishops, sufferers were cast out through formal religious separation ceremonies symbolising that they were legally dead to the world.

Segregated victims were forced into permanent **exile to isolated Lazar houses** established outside city walls. Sufferers were legally forbidden from entering narrow town streets, touching healthy children, or drinking from public wells. To prevent contagion, lepers were compelled to wear distinctive grey cloaks, sounding **warning wooden clappers & bells** to alert passing travellers of their approach along rural parish roads.

ENQUIRY 01 • PART II

TURNING POINTS & HISTORICAL CONSEQUENCES

Teleology & The Papal Monopoly on Knowledge

DECISIVE HISTORICAL MECHANISM Questioning Galen became synonymous with heresy against God, freezing anatomical progress for over 1,300 years.

[1.3] Astrological Influences and the Zodiac Man

"How did the Church reconcile astrology with Christian doctrine?"

Alongside divine retribution, medieval society relied heavily on astrology to explain the unpredictable timing and severity of disease. Drawing upon translated Arabic scholarship, medieval physicians believed celestial movements governed human anatomy. Astrological charts such as **Zodiac Man parchment wheels** mapped zodiac constellations directly onto human organs, teaching that the Moon, Mars, and Saturn controlled internal organs, humours, and vital blood circulation.

Astrological timing was considered essential before performing physical treatments like bloodletting or surgery. When the catastrophic Black Death struck Europe, university scholars blamed the planetary crisis on the infamous **1345 planetary conjunction**, arguing that an unholy **Mars, Saturn & Jupiter alignment** in the sign of Aquarius had drawn corrupt poisonous vapors from the earth to infect the atmosphere.

[1.4] Spiritual Intercession, Relics, and Miracle Cures

"Why did sick people turn to pilgrimage shrines rather than doctors?"

Because disease was viewed as supernatural in origin, treatments were predominantly religious and spiritual. Suffering patients undertook **arduous pilgrimage to Canterbury** to pray at the sacred tomb of Archbishop Thomas Becket, or travelled to Walsingham. Pilgrims sought miraculous healing through **kissing holy relics of saints**, believing that touching holy bones, dried blood, or fragments of the True Cross channeled miraculous divine grace.

Wealthy medieval families secured spiritual protection by funding perpetual **chantry masses & sale of indulgences**, paying parish priests to recite prayers for sick relatives to hasten healing or purge sins. Monasteries and cathedrals organized solemn processions carrying jeweled reliquaries through crowded streets during pestilence, demonstrating how medieval healing remained an act of collective religious devotion rather than clinical intervention.

[1.5] Monastic Scriptoria, Galen and Church Censorship

"Why did church enforcement of Galen prevent medical progress?"

While the Catholic Church preserved ancient medical manuscripts through **monastic scriptoria Latin copying**, it strictly policed intellectual inquiry. Church authorities enthusiastically embraced the classical writings of Galen because his teachings promoted a single divine creator, turning **Galen purposeful creator dogma** into indisputable theological truth. Questioning Galen's anatomical accuracy was condemned as dangerous heresy that challenged God's perfect design of the human body.

The Church enforced absolute intellectual conformity across medical universities, fiercely suppressing empirical scientific investigation. When English philosopher **Roger Bacon imprisoned for heresy (1277)** by church superiors after advocating direct scientific experiment and observation over ancient texts, scholars were intimidated into silence. Consequently, medical education remained frozen for over two centuries, prioritizing rote recitation of ancient Latin authorities over original clinical research.

ENQUIRY 02 • PART I

PEARSON EDEXCEL GCSE (9–1) HISTORY • PAPER 1 (1HI0/11)

Rational Explanations: Hippocrates, Galen & The Four Humours (c1250–c1500)

KEY ENQUIRY QUESTION *How did the Theory of the Four Humours provide a logical, complete explanation for every illness in medieval England?*

[2.1] Hippocrates and the Secular Shift in Medicine

"How did Hippocrates separate medicine from superstition?"

In fifth-century BC Greece, Hippocrates revolutionized healing by pioneering the **Hippocratic clinical method**. Instead of attributing illness to divine wrath or demonic possession, he championed careful bedside observation, recording patient symptoms, pulse, and fevers systematically. This revolutionary approach represented a dramatic **rejection of gods** and supernatural superstition, establishing the enduring principle that physical disease was caused by natural bodily imbalances.

Hippocrates also established medical professionalism through the **Hippocratic Oath ethics**, which bound physicians to preserve life, protect patient confidentiality, and do no deliberate harm. His extensive written treatises formed the foundation of Western rational medicine. When translated into Latin during the Middle Ages, his secular diagnostic principles became the core curriculum taught in newly founded European university medical faculties.

[2.2] The Four Humours and Humoral Equilibrium

"Why did humoral theory seem logical to medieval people?"

The cornerstone of classical rational medicine was the Theory of the **Four bodily humours**, developed by Hippocrates and expanded by Galen. According to this theory, the human body comprised four essential fluids: **blood, phlegm & biles** (yellow bile and black bile). Good health required an exact equilibrium of these humours, corresponding directly to the four universal elements of air, fire, water, and earth.

Humoral balance fluctuated according to individual temperament, diet, age, and a complex **seasonal balance system**. For example, wet and cold winter seasons increased phlegm, causing coughs, while hot summer weather elevated dry yellow bile, causing fevers. When humours became corrupted or unbalanced, illness resulted, providing medieval doctors with a comprehensive, rational framework to diagnose ailments without reference to supernatural intervention.

ENQUIRY 02 • PART II

TURNING POINTS & HISTORICAL CONSEQUENCES

The Humoural Axis: Balance, Phlebotomy & Opposites

DECISIVE HISTORICAL MECHANISM Humoural medicine was rational because it sought natural biological causes rather than demonology, yet remained medically ineffective.

[2.3] Galen's Theory of Opposites and Animal Dissections

"How did the Theory of Opposites provide practical treatment rules?"

In second-century Rome, Claudius Galen expanded Hippocratic humoralism by developing the influential **Galen Theory of Opposites**. While Hippocrates relied on passive rest, Galen introduced active treatment based on the principle that **contraries cure contraries**. For instance, a cold, moist fever was aggressively treated with hot, dry remedies like crushed pepper and wine, while hot inflammatory fevers were treated with cooling cucumber.

Galen also produced extensive anatomical treatises, but because Roman religious law strictly prohibited human dissection, he relied on pigs, dogs, and apes. This led to serious **animal dissection errors**, including claiming the human jawbone comprised two bones and that blood passed through invisible heart septum pores. Canonized by the medieval Church, Galen's anatomical mistakes remained unchallenged for fourteen centuries.

[2.4] Diagnostic Uroscopy: The Matula and the Urine Wheel

"Why did uroscopy give physicians great diagnostic authority?"

Medieval physicians commanded immense diagnostic prestige without ever performing surgery or physically touching their wealthy patients. The primary clinical tool was the **uroscopy urine flask (matula)**, a pear-shaped glass vessel designed to resemble the human bladder. Physicians held the matula up to the light, closely scrutinizing the urine sample's clarity, sediment, density, and even tasting drops to detect sweet or acidic imbalances.

The physician cross-referenced findings against a colorful **urine diagnostic wheel**, matching twenty distinct shades ranging from white to tawny, red, and black to determine which internal humour was corrupted. Finally, physicians checked planetary positions for **Zodiac Man alignment** before prescribing treatments, ensuring that bloodletting or purging occurred only under favorable astrological auspices to avoid exacerbating internal humoral corruption.

[2.5] Miasma Theory and Foul Environmental Vapours

"Why did rational physicians link bad smells directly to epidemics?"

To explain why thousands of citizens perished simultaneously during epidemic outbreaks, rational ancient and medieval doctors formulated the **miasma theory (bad air)**. Originating in classical Greece, this theory taught that infectious pestilence was generated when poisonous fumes rose from rotting animal corpses, stagnant swamps, unburied bodies, and decaying organic waste, contaminating the surrounding atmosphere with invisible airborne toxins.

Medieval urban life intensified **cesspool stench fears**, as open sewage, butchery offal, and rotting waste clogged unpaved town streets. Breathing this foul, pestilential air was believed to poison vital spirits inside the lungs, causing acute **unbalanced bodily fluids** and rapid humoral decay. This environmental explanation encouraged municipal councils to clear street dung and burn fragrant tar barrels during epidemics to purify the air.

ENQUIRY 03 • PART I

Approaches to Prevention & Treatment: Rituals, Bleeding & Purging (c1250–c1500)

KEY ENQUIRY QUESTION *Why did standard medieval medical treatments frequently endanger the lives of the patients they were meant to cure?*

[3.1] Spiritual Remedies: Prayer, Penance and Processions

"Why did ordinary people consider prayer more vital than medicine?"

Before administering physical medicines, medieval patients sought divine reconciliation to restore spiritual grace. Because sickness was interpreted as a trial or punishment sent by God, religious treatments were mandatory. Communities engaged in devout **penance & fasting**, while monastic clergy recited continuous **chanted Latin prayers** and held special intercessory masses beside sickbeds, imploring saints to intercede and beg God for physical and spiritual deliverance.

During catastrophic epidemics like the 1348 Black Death, religious panic reached extreme heights across Europe. Believing the apocalypse was at hand, radical religious sects formed **flagellant whipping processions**, marching barefoot between towns while scourging their bare backs with iron-tipped whips to appease God's wrath. Although extreme, flagellation demonstrated how deeply ordinary medieval people believed that spiritual penance was the only true cure.

[3.2] Phlebotomy: Restoring Humoral Balance by Bloodletting

"Why was bloodletting the most universal physical treatment?"

The most widespread rational physical treatment in medieval medicine was bloodletting, or phlebotomy. Humoral pathology taught that excess blood caused inflammation, fever, and violent disease. Surgeons performed **phlebotomy vein incision** using specialized iron blades, or applied aquatic **iron fleams & leeches** to delicate skin, allowing these blood-sucking worms to extract corrupted fluid until the patient turned pale or faint.

Another popular bloodletting method was dry or wet cupping, using heated **cupping & pewter bowls** to create a vacuum over scarified skin, drawing blood to the surface. Barber-surgeons collected the extracted blood into measured pewter bowls to inspect its color, thickness, and smell. Bloodletting was administered routinely as preventative healthcare every spring, leaving millions of medieval patients dangerously weakened.

ENQUIRY 03 • PART II

TURNING POINTS & HISTORICAL CONSEQUENCES

Regimen Sanitatis: Preventive Hygiene & The Six Non-Naturals

DECISIVE HISTORICAL MECHANISM Medieval people actively cared about hygiene; sickness was prevented by keeping the internal humours and external environment pure.

[3.3] Purging, Emetics and Clysters: Evacuating Corrupt Humours

"How was purging believed to restore bodily equilibrium?"

When phlebotomy was deemed insufficient to balance humours, physicians prescribed systemic purging to cleanse the digestive tract. To induce violent vomiting, doctors administered powerful **emetics (black hellebore)**, bitter herbs, and antimony potions that forced the stomach to expel toxic yellow bile. These caustic herbal concoctions caused agonizing abdominal cramps and dehydration, yet were celebrated for cleansing internal impurities.

To purge the lower intestines, medical practitioners administered herbal **laxatives & clysters** (enemas). Using a hollow pewter pipe attached to an oiled pig's bladder, barber-surgeons injected warm water, bitter herbs, and crushed chamomile deep into the rectum. This aggressive **humoural fluid evacuation** was intended to expel black bile and putrid matter, reflecting the core medieval belief that expelling fluids restored internal bodily harmony.

[3.4] Herbal Remedies, Simples and the Great Theriac

"Why did herbal recipes remain popular across all social classes?"

When treating non-fatal everyday illnesses, medieval patients turned to herbal pharmacopeia drawn from classical manuscripts like Dioscorides' **Herbal Materia Medica**. Healers gathered local medicinal plants, preparing herbal teas, soothing honey syrups, and warm poultices from mint, chamomile, sage, and plantain. For ordinary peasant households, these traditional botanical remedies provided affordable, comforting relief for coughs, cuts, joint aches, and digestive discomfort.

For wealthy elites, apothecaries compounded the legendary universal antidote **Theriac (70 ingredients)**, which blended crushed snake flesh, opium, exotic Arabian spices, and wine. Sold as expensive **local apothecaries remedies**, theriac was believed to neutralize internal poisons and ward off pestilence. While herbs like willow bark offered mild anti-inflammatory relief, most complex medieval compounds possessed zero clinical power against virulent bacterial infections.

[3.5] The Regimen Sanitatis and Preventative Health

"How did wealthy barons attempt to prevent sickness before it struck?"

Alongside curative treatments, medieval medicine emphasized preventative lifestyle moderation codified in the widely translated Salernitan **Regimen Sanitatis rules**. Originating from the medical school at Salerno, this guide instructed citizens to balance six essential factors: diet, exercise, sleep, digestion, fresh air, and emotional tranquility. Moderation was key: overeating, sudden rage, and excessive sleeping were believed to destabilize delicate humoral equilibrium.

Because miasma was feared as a major cause of disease, medieval prevention focused on environmental cleansing. Wealthy townspeople carried **sweet pomander scents**, holding hollow silver orbs filled with fragrant ambergris and cloves to their noses. Citizens also purified indoor air by burning **lavender & burning tar** barrels in streets to counteract foul atmospheric stenches, illustrating rational attempts to ward off airborne infection.

ENQUIRY 04 • PART I

PEARSON EDEXCEL GCSE (9–1) HISTORY • PAPER 1 (1HI0/11)

Medical Care Providers & Monastic Hospitals: 'Care Not Cure' (c1250–c1500)

KEY ENQUIRY QUESTION *How did social hierarchy and the Catholic Church shape who provided medical care in medieval England?*

[4.1] University-Trained Physicians: Theory Over Practice

"Why were university physicians accessible only to the ultra-wealthy?"

Medieval healthcare in England was strictly stratified by social wealth. At the top of the professional medical hierarchy stood elite **university-trained physicians**, educated at Oxford, Cambridge, Paris, or Bologna. These doctors spent up to ten years studying ancient **Latin texts & star charts**, mastering Galenic humoral theory and astrological almanacs without ever touching a patient or receiving practical surgical instruction.

Because few university physicians practiced in England—fewer than one hundred by 1300—they charged exorbitant **high diagnostic fees**, catering exclusively to the royal court, wealthy bishops, and noble aristocrats. A physician evaluated uroscopy samples, consulted astrological charts, and wrote Latin prescriptions, but considered manual surgical labor beneath their professional dignity, delegating physical operations to practical barber-surgeons.

[4.2] Guild Apothecaries and Spicers: Retail Medicine

"How did apothecaries provide accessible medicines to townspeople?"

For urban merchants and craftspeople who could not afford a university physician, the local apothecary shop was the first port of call. Operating under strict **apothecary guild trade** regulations, apothecaries trained through rigorous apprenticeships rather than academic degrees. They possessed detailed practical knowledge of botanical chemistry, compounding simples, electuaries, mineral powders, and medicinal syrups imported from Mediterranean and Eastern trade routes.

In their shops, diverse **herbal preparations sold** alongside spices, preserved fruits, and scented candles. Although physicians viewed apothecaries with professional suspicion, accusing them of prescribing drugs without diagnostic training, apothecaries flourished by providing **cheaper urban access** to medicine. Ordinary townspeople frequently consulted apothecaries directly, purchasing inexpensive herbal purgatives, warming chest cordials, and teething balms without paying an expensive physician's consultation fee.

ENQUIRY 04 • PART II

TURNING POINTS & HISTORICAL CONSEQUENCES

'Care Not Cure': The Monastic Hospital Philosophy

DECISIVE HISTORICAL MECHANISM "Care Not Cure" defines the institutional purpose of all 500+ medieval English monastic hospitals.

[4.3] Barber-Surgeons and Practical Craft Medicine

"Why were barber-surgeons viewed as lower-status craftsmen?"

For physical trauma, wound care, and operative procedures, medieval townspeople relied on the practical craftsmen of the **barber-surgeon guild**. Ranked far below physicians in social status, barber-surgeons served apprenticeships and were associated with barbers because both professions wielded sharp razors. They performed everyday minor surgery, providing **bloodletting & tooth pulling**, lancing painful boils, and stitching battlefield flesh wounds.

Barber-surgeons also tackled dangerous emergency operations, including basic **cauterisation & amputation** of crushed limbs and removing bladder stones using specialized iron scoops. Without anesthetics or antiseptic knowledge, patients were held down by assistants while surgeons worked with raw speed. Wounds were sealed with boiling elderberry oil or red-hot irons, resulting in immense shock, agony, and high mortality rates from postoperative hospital gangrene.

[4.4] Monastic Hospitals: The Philosophy of 'Care Not Cure'

"Why did monastic hospitals deliberately avoid medical treatment?"

By 1500, England possessed over seven hundred hospitals, virtually all founded, funded, and operated by Christian religious orders as **monastic charity hospitals**. Institutions such as St Bartholomew's and St Thomas' in London were run by Catholic monks and nuns. Rather than treating contagious diseases, hospitals provided refuge for travelers, elderly pilgrims, the blind, and the dying, offering food and shelter.

Medieval hospitals operated strictly under a **care not cure philosophy**, viewing spiritual salvation as far more important than physical bodily healing. Patients were kept warm, given clean linen, fed wholesome soups, and surrounded by quiet **bed rest & prayer**. Chapels were positioned so bedridden inmates could observe the elevation of the Host, preparing their souls for heaven rather than undergoing medical surgery.

[4.5] Domestic Women Healers and Household Medicine

"Why did the vast majority of sick people rely on female family members?"

Over ninety percent of England's medieval population lived in rural villages, far from university physicians or town guild apothecaries. For these ordinary families, healthcare was delivered almost entirely by **domestic women healers**, including mothers, grandmothers, and local village wise women. Women held responsibility for family health, preparing remedies from handwritten recipe books (*leechbooks*) passed down through generations of rural homemakers.

Relying on extensive **local herb gardening**, women grew sage, chamomile, mint, and elderberry to brew soothing fever teas, poultices, and cough syrups. Wise women also performed vital **midwifery & home care**, delivering newborn infants, caring for the elderly, and nursing bedridden neighbours. Despite delivering practical bedside care for centuries, their position remained vulnerable to accusations of witchcraft if treatments failed to cure.

ENQUIRY 05 • PART I

Case Study: Dealing with the Black Death (1348–1349)

KEY ENQUIRY QUESTION *Why were medieval communities, doctors, and civic authorities completely powerless to stop the Black Death?*

[5.1] The Pestilence Arrives: Maritime Trade and Melcombe Regis (1348)

"How did trade routes unwittingly bring the plague to England?"

In the summer of 1348, the Second Plague Pandemic struck England through international maritime commerce. A merchant cog from Gascony dropped anchor at **Melcombe Regis port (1348)** in Dorset, carrying a lethal cargo across the English Channel. Within the ship's hold, cargo crates carried the microscopic **Yersinia pestis bacteria**, harbored within the blood of black rats and transmitted by parasitic tropical rat fleas.

When dockers unloaded wool sacks and grain baskets onto wharves, infected **black rats & fleas** escaped into crowded port alleyways. Medieval timber dwellings, open rubbish heaps, and unpaved streets provided the ideal environment for the rodent population to multiply rapidly. Within weeks, the infection established an unstoppable foothold, spreading inland along river trade routes and reaching London by autumn 1348.

[5.2] The Dual Pathology: Bubonic Swellings and Pneumonic Terror

"Why was the pneumonic strain far deadlier than the bubonic form?"

The Black Death presented in two lethal clinical manifestations. In its bubonic form, flea bites injected plague bacteria directly into the human lymphatic system, producing agonizing **bubonic groin swellings (buboes)**, blackened neck tumors, and armpit sores that oozed foul pus and blood. Sufferers experienced raging fevers, violent vomiting, dark subcutaneous blotches, and unbearable muscle spasms before falling into comatose states.

Far more terrifying was the pneumonic form, which occurred when bacteria infected the lungs. Victims began **pneumonic bloody coughing**, aerosolizing bacteria directly into shared room air and infecting entire households through breathing. The virulence was devastating, with **mortality in three days** following initial fever, killing nearly one hundred percent of pneumonic victims and leaving entire peasant families dead in their timber cottages.

ENQUIRY 05 • PART II

TURNING POINTS & HISTORICAL CONSEQUENCES

Astrology, Miasma & Contagion: The 1348 Conjunction

DECISIVE HISTORICAL MECHANISM Medieval authorities attempted practical public health measures, but lacked biological knowledge of flea vectors.

[5.3] Desperate Cures: Live Hens, Bloodletting and Crushed Emeralds

"Why were medical treatments completely useless against the plague?"

Lacking understanding of bacterial pathology, medieval medical treatments proved completely futile against the plague. Convinced the pandemic was caused by **divine wrath & miasma**, healers tried sympathetic remedies to draw poison from the body. Folk practitioners recommended **lancing buboes & live hens**, strapping plucked chickens or warm dried toads directly against swollen groin sores, which only caused agonizing pain, blood poisoning, and septicaemia.

Qualified physicians relied on aggressive **bleeding & crushed emeralds**, draining pints of blood from weakened patients and feeding wealthy nobles expensive potions containing crushed gemstones dissolved in vinegar. Concurrently, patients swallowed powdered theriac containing snake venom, while religious zealots engaged in public flagellation. None of these desperate treatments possessed any pharmacological efficacy, and mortality rates remained catastrophic across all social classes.

[5.4] Quarantine, Butchery Orders and Environmental Cleansing

"How did belief in miasma shape royal prevention orders?"

As death rates soared, municipal authorities attempted emergency public health measures. In Gloucester and London, town councils introduced rudimentary **quarantine isolation rules**, shutting infected families inside their houses and barring visitors from affected neighboring parishes. However, because authorities had no concept of flea-borne transmission, isolating humans inside flea-infested timber dwellings actually ensured that entire trapped families perished together in quarantine.

Attributing the epidemic to corrupt miasma, King Edward III sent urgent letters demanding **cleaning town butchery waste** from London streets, ordering butchers to stop dumping offal and animal entrails into the River Thames. Furthermore, councils constructed continuous **street fire smoke barriers**, burning pitch, sulfur, and sweet herbs at crossroads day and night to purify the corrupt atmosphere, though these measures failed to stop plague fleas.

[5.5] Demographic Collapse: Mass Graves and Feudal Transformation

"Why did the Black Death trigger a long-term social and economic crisis?"

By winter 1349, the immense volume of dead overwhelmed churchyards. Authorities purchased suburban fields, excavating deep **mass burial trenches** at East Smithfield outside London walls, where corpses were stacked five deep in rough timber coffins. Between 1348 and 1350, between a **third to half population lost**, wiping out an estimated 1.5 to 2 million people across England and leaving entire villages completely abandoned.

This demographic catastrophe permanently transformed medieval feudal society. The sudden annihilation of agricultural workers generated an acute **labour shortage & wage rises**, empowering surviving peasants to demand higher wages and freely leave their lords' manors. When Parliament attempted to freeze wages through the 1351 Statute of Labourers, deep peasant resentment erupted, fatally undermining feudal servitude and culminating in the 1381 Peasants' Revolt.

Master Knowledge Retrieval Quiz

Medieval Medicine in Britain • c1250–c1500 (30 Essential Recall Questions)

Standard Recall Protocol: Partner Quiz or Solo Practice. Test your rapid chronological, factual, and conceptual recall across all 5 enquiries. All questions are cross-referenced directly to paragraph locations in this textbook; zero answers are printed to maintain active retrieval integrity.

Enquiry 01: Supernatural & Religious Expla... pp. 2–3 • Ref [1.1]–[1.5]

1. Which international institution controlled all education, book copying, and hospital foundations in medieval England?
2. What term describes the system of land ownership, loyalty, and social hierarchy established across England after 1066?
3. Why did the vast majority of ordinary medieval peasants lack direct access to Latin medical manuscripts?
4. What language was universally used across medieval Western Europe for church services, legal documents, and university lectures?
5. What ancient European empire collapsed around AD 476, leading to the fragmentation of public health infrastructure in Britain?

Enquiry 02: Rational Explanations: Hippocr... pp. 4–5 • Ref [2.1]–[2.5]

6. Who controlled formal medical education and the copying of books in medieval England?
7. Why was sudden illness viewed as a spiritual condition rather than a biological infection?
8. How did medieval society treat leprosy sufferers based on biblical scripture?
9. What celestial event in March 1345 did University of Paris scholars blame for the Black Death?
10. What pocket manual did medieval physicians carry to consult astrological diagrams and urine charts?

Enquiry 03: Approaches to Prevention & Tr... pp. 6–7 • Ref [3.1]–[3.5]

11. Which ancient Greek physician originally developed the Theory of the Four Humours?
12. Which of the four humours was associated with spring, air, and being hot and wet?
13. How did Galen's Theory of Opposites suggest treating a patient with cold, wet phlegm?
14. What was miasma believed to be in medieval medicine?
15. What was a glass matula used for in medieval clinical diagnosis?

Enquiry 04: Medical Care Providers & Mon... pp. 8–9 • Ref [4.1]–[4.5]

16. What was the most common surgical treatment performed in medieval England to restore humoural balance?
17. Which instrument was commonly used to open a patient's vein during bloodletting?
18. What method of bloodletting involved creating a warm vacuum over scratched skin?
19. What was Theriac in medieval medicine?
20. Why did physicians prescribe emetics and violent laxatives to patients?

Enquiry 05: Case Study: Dealing with the B... pp. 10–11 • Ref [5.1]–[5.5]

21. Where were university-trained physicians educated in medieval Europe?
22. Why were university-trained physicians socially superior to barber-surgeons?
23. What manual procedures did guild-trained barber-surgeons perform?
24. What did apothecaries do in the medieval medical division of labour?
25. Who provided the vast majority of day-to-day healthcare for ordinary medieval peasants?

Synoptic Synthesis & Historical Causation pp. 2–11 • Whole-Era Recall

26. **[Synoptic]** Explain the single biggest obstacle to medical progress across this era.
27. **[Synoptic]** Which institution or technological breakthrough triggered the greatest shift in clinical practice?
28. **[Synoptic]** How did contemporary beliefs about the causes of disease influence the treatments administered?
29. **[Synoptic]** Assess the extent of continuity between the start and end of this chronological period.
30. **[Synoptic]** Identify one individual whose contributions laid the groundwork for subsequent breakthroughs.